

HOW TO OPEN A NON-MEDICAL HOME CARE AGENCY in Texas

A licensed HCSSA (Personal Assistance Services) — private pay, Medicaid, VA & long-term-care insurance.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The Texas model — read this first

In Texas, an agency that sends caregivers into people's own homes to help with daily living — bathing, dressing, meals, light housekeeping, companionship — is licensed by the Texas Health and Human Services Commission (HHSC) as a Home and Community Support Services Agency (HCSSA) in the Personal Assistance Services (PAS) category. This is NON-MEDICAL care: help with the activities of daily living, not skilled nursing. You don't need a building or a nurse-owner — you need the HCSSA license, an administrator, and a way to recruit and supervise attendants. This guide walks the whole path — licensing the agency, then getting paid — broken down so a first-timer can follow it.

You can start earning on private pay while Medicaid enrollment catches up. A big advantage of non-medical home care: once you hold the HCSSA-PAS license you can serve PRIVATE-PAY clients immediately — no Medicaid enrollment, no Electronic Visit Verification, no MCO contract required. Medicaid (through STAR+PLUS and the attendant programs) is a larger, steadier market, but it takes months to enroll and contract. Most successful agencies launch on private pay and long-term-care insurance, then add Medicaid. Part 8 covers both.

Accreditation — optional here, but it opens doors. Texas does NOT require national accreditation to hold a PAS license or bill Medicaid — the state license is what lets you operate. But accreditation (ACHC, CHAP, or The Joint Commission) is often required by managed-care plans, VA contracts, long-term-care insurers, and franchise brands, and it's the on-ramp if you later add skilled home health. Every PolicyReady product is built to those standards behind the scenes; treat accreditation as a goal for year two (see Part 9).

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It is written for someone who has never opened an agency — where we name a thing (an agency, a form, a rule, an insurance type), we break it down and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; verify each step with HHSC, TMHP, and qualified counsel (see Part 11) before you rely on it.

The journey at a glance

Ten milestones take you from idea to a licensed, operating, paid agency. Each part below is one rung.

1	Decide this is your business	A licensed non-medical home-care agency — recurring, hourly revenue (Part 1)
2	Form the business & insure	Entity, EIN, NPI, and the right insurance & bonding (Part 3)
3	Get the HCSSA-PAS license	Pre-survey training, TULIP application, the fee (Part 4)
4	Set up your administrator	The 24-hour administrator training + alternate administrator (Part 5)
5	Recruit & screen attendants	Service supervisor, competency, EMR/NAR & criminal checks (Part 5)

6	Set up EVV & policies	HHAeXchange EVV, P&P Manual, your first-client survey (Part 6)
7	Serve private-pay clients	Start earning while Medicaid enrollment is pending (Part 8)
8	Enroll for Medicaid	TMHP/PEMS + contract the STAR+PLUS health plans (Part 8)
9	Get your first clients	Discharge planners, home-health/hospice partners, AAAs (Part 10)
10	Who to call & what to ask	Every contact + the exact questions, all in one place (Part 11)

Rule of thumb. Do the free HHSC pre-survey training and get your entity in good standing with the Comptroller and Secretary of State early — the license won't issue without both. Start on private pay so you're generating revenue while the months-long Medicaid enrollment and MCO contracting run in the background.

PART 1

What This Business Is

In short: A licensed HCSSA in the Personal Assistance Services category — caregivers who help clients with daily living in their own homes. Non-medical, agency-based, paid hourly.

Texas licenses agencies that deliver care in the home as Home and Community Support Services Agencies (HCSSAs), under Health & Safety Code Chapter 142 and 26 Texas Administrative Code Chapter 558. An HCSSA picks one or more license categories; a non-medical agency picks Personal Assistance Services (PAS). PAS is defined as routine, ongoing care that helps a person with the activities of daily living (ADLs — bathing, dressing, grooming, eating, toileting, mobility) and the functions of daily living (IADLs — meals, light housekeeping, laundry, errands, companionship).

PAS (non-medical) vs. skilled home health

	Personal Assistance Services (PAS)	Licensed Home Health (skilled)
What it is	Help with daily living by attendants	Nursing, therapy, wound care by licensed clinicians
Who delivers it	Trained attendants/caregivers	RNs, LVNs, PT/OT/ST
Medications	Generally cannot administer meds or fill pill boxes	Nurses administer/manage medications
Your license	HCSSA — PAS category	HCSSA — Licensed Home Health category

This guide is for the PAS category. If you ever want to add skilled nursing, that's a different (larger) license category — you can add it later.

The one clinical line to respect. A PAS agency's attendants generally may NOT administer medications or fill a pill box — those are nursing tasks. Attendants can remind a client to take their own medication and hand them the bottle. Certain health-related tasks can be delegated to an attendant by a registered nurse under Texas Board of Nursing delegation rules, but that requires RN involvement. Keep your scope clear so you stay inside the PAS license.

Key terms. HHSC = Texas Health & Human Services Commission (your licenser). HCSSA = Home and Community Support Services Agency (the license). PAS = Personal Assistance Services (your category). TULIP = the online licensing portal. EVV = Electronic Visit Verification (required for Medicaid visits). STAR+PLUS = Texas Medicaid managed long-term care. MCO = Managed Care Organization (a STAR+PLUS health plan). CFC = Community First Choice. CDS = Consumer Directed Services (the client-as-employer option). FMSA = Financial Management Services Agency. TMHP = Texas Medicaid & Healthcare Partnership. PEMS = the Medicaid provider-enrollment system. EMR/NAR = Employee Misconduct Registry / Nurse Aide Registry. NPI = National Provider Identifier.

Why this business — and why now

Demand for in-home care is rising fast, and for reasons that aren't going to reverse: people overwhelmingly want to age in their own homes rather than move to a facility; the 65-and-over population grows every year; and hospitals now discharge patients “quicker and sicker,” which sends more families looking for help at home right after a hospital stay. Texas has one of the largest and fastest-growing 65-and-over populations in the country — millions of older adults, swelling every year — which means a deep, durable pool of families who need and can arrange in-home care. And it's a business you can start lean — there's no building to buy or fill, the revenue is recurring and hourly, and a good reputation compounds through referrals.

The three mistakes that sink new agencies — avoid these. New home-care owners rarely fail because of the care itself. They fail for three business reasons: (1) undercapitalization — they run out of cash covering weekly payroll before Medicaid or insurers reimburse (see the cash-flow note in Part 9); (2) weak caregiver recruiting and retention — they can't reliably staff the shifts they're offered, so referral sources quietly stop calling; and (3) sloppy compliance — a missing background check, training record, or (for Medicaid) EVV visit that triggers a payback or a survey finding. Get your cash runway, your staffing pipeline, and your paperwork right, and the rest follows.

PART 2

Who Regulates You (and What Each One Does)

In short: HHSC licenses and surveys you, TMHP + the STAR+PLUS health plans pay you for Medicaid clients, the Texas Board of Nursing governs any delegated tasks, and EVV verifies Medicaid visits.

HHSC — Health and Human Services Commission (your licensor)

HHSC, through its Long-Term Care Regulation / Health Care Facilities Regulation area, issues your HCSSA license, writes the 26 TAC Chapter 558 standards, runs the TULIP application system, and conducts the post-license and ongoing surveys. Provider information is on hhs.texas.gov (Home and Community Support Services Agencies).

TMHP and the STAR+PLUS health plans (who pay you for Medicaid clients)

The Texas Medicaid & Healthcare Partnership (TMHP) is where you enroll as a Medicaid provider (through the PEMS system). Most Medicaid personal care runs through STAR+PLUS, so you then contract with the STAR+PLUS Managed Care Organizations (MCOs) serving your area, and each member's MCO service coordinator authorizes the hours. TMHP Contact Center: 800-925-9126.

The Texas Board of Nursing (for delegated tasks)

If an attendant will perform a health-related task that would otherwise be nursing (beyond basic ADL help), a registered nurse must delegate and supervise it under the Texas Board of Nursing's delegation rules. For pure companion/personal-care work this doesn't come up, but know the line — it's what separates PAS from skilled care.

EVV — Electronic Visit Verification

For Medicaid personal-care visits, Texas requires Electronic Visit Verification — an electronic record of who provided care, for whom, where, and when. Texas uses HHAeXchange as its free statewide EVV system (or an HHSC-approved alternative), and a claim must match a verified EVV visit before it's paid (Part 6). Private-pay-only work is not subject to EVV.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In Texas you form the LLC at the Secretary of State, and there's no state income tax — but you'll register with the Texas Comptroller and file an annual Public Information Report (and any franchise tax) to keep the entity in good standing, which HHSC checks before it licenses you. You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. Go to the Texas Secretary of State.** File online through SOSDirect/SOSUpload at sos.state.tx.us, or by mail. Main business line (512) 463-5555.
- 2. File a Certificate of Formation — LLC (Form 205).** The filing fee is \$300 (credit-card payments add a 2.7% convenience fee). Name the LLC and list a registered agent with a physical Texas address (can be you).
- 3. Adopt an Operating Agreement.** The internal document setting out ownership, management, and money. Texas doesn't file it, but your bank and partners will want it.
- 4. Get right with the Comptroller.** Register with the Texas Comptroller; you'll file an annual Public Information Report (most small agencies owe no franchise tax, since the 2026 no-tax-due revenue threshold is \$2.65 million, but you still file). HHSC checks that your entity is in good standing before it licenses you.

No newspaper publication in Texas. Unlike some states, Texas does not make you publish a formation notice. If you operate under a name different from the LLC's legal name, file an Assumed Name Certificate (DBA).

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

- 1. Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You'll use it to enroll with Texas Medicaid through TMHP and bill the STAR+PLUS plans (Part 8). Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.
- 2. Start a Type 2 (Organizational) application.** Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.

3. Enter your legal name, EIN, and address, and pick your taxonomy. Choose the personal care services taxonomy (3747P1801X) that fits a home-care agency. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.

4. Submit — it's free and quick. It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a major bank with lots of Texas branches — Chase, Bank of America, Wells Fargo, Frost Bank (a large Texas-based bank), or a local credit union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

A home-care agency sends employees into clients' homes, so your risk is different from a facility's — you need coverage built for that:

Coverage	What it protects against	Must-have?
General Liability (GL)	Bodily injury/property damage claims arising from operations	Yes — foundational
Professional Liability (E&O)	Claims about the CARE — a caregiver's error or neglect	Yes — top coverage
Abuse & Molestation	Allegations of abuse by a caregiver (GL excludes this!)	Yes — high exposure
Employee Dishonesty / Surety Bond	Theft from a client's home by an employee	Yes — clients & payers expect it
Non-Owned & Hired Auto	Caregivers driving their own cars for/with clients	Yes — critical for home care
Workers' Compensation	Caregiver injuries (lifting/transferring is high-risk)	Optional in TX, but strongly advised
Cyber Liability	A breach of client health information (HIPAA)	Strongly recommended

Two coverages home-care owners forget. (1) *Non-owned/hired auto* — your caregivers drive their own cars to clients and sometimes drive clients to appointments; without this, an accident on the job can land on you. (2) *A caregiver dishonesty bond* — clients and referral sources will ask whether your caregivers are “bonded.” Get both from the start.

Real places to get it

Buy through an independent agent/broker who writes home-care programs. Carriers/programs active in this niche include Philadelphia Insurance (PHLY), CNA, The Hartford, Markel, and home-care specialty programs from Caitlin Morgan, Gallagher, NIP Group/HomeCareUSA, and Combined/CalSurance. To find a local Texas agent, use [trustedchoice.com](https://www.trustedchoice.com) (filter home care/home health), or ask a franchise or peer. Verify each carrier writes non-medical home care in Texas before relying on a quote.

What to ask the insurance agent

“Do you write NON-MEDICAL home care (PAS) — which carriers and what's the minimum premium?”

“Is professional liability included with GL, and is abuse & molestation included or a sub-limit?”

“Do you include non-owned & hired auto, and an employee-dishonesty bond?”

“What limits do the STAR+PLUS MCOs, the VA, or long-term-care insurers require to contract?”

“Should I carry workers' comp, or is a non-subscriber plan smarter for my payroll?”

PART 4

Get Your HCSSA – PAS License — Step by Step

In short: Do the free HHSC pre-survey training, apply in TULIP with your documents and the fee, get licensed, then request your initial survey once you have your first client. PAS agencies are surveyed AFTER licensure.

4.1 Before you apply

- ☐ Entity formed, in good standing with the Secretary of State and the Comptroller
- ☐ EIN and Type 2 NPI obtained; insurance & bond bound
- ☐ A qualified administrator and alternate administrator identified (Part 5)
- ☐ Your policies & procedures written (your P&P Manual) and forms ready
- ☐ The free HHSC HCSSA Pre-Survey Computer-Based Training (Modules 1–5) completed

4.2 The application (TULIP)

- 1. Complete the pre-survey CBT first.** HHSC requires the free online HCSSA pre-survey training (Modules 1–5) before licensure — save your completion record.
- 2. Apply in TULIP.** The Texas Unified Licensure Information Portal is where you file. A parent PAS agency uses HHSC Form 2021 (branch/alternate-site forms differ).
- 3. Choose the PAS category.** Select Personal Assistance Services as your license category.
- 4. Pay the fee.** An initial 3-year HCSSA license is \$2,625 for a parent agency (an alternate delivery site is \$1,000). Confirm the current amount on the HHSC fee schedule.
- 5. Upload your documents** and confirm your Comptroller/SOS good standing; HHSC has up to 45 days to process a complete application (you get 30 days to cure any deficiency).

PAS agencies are surveyed AFTER you're licensed — a real advantage. Unlike a facility that must pass inspection before opening, a PAS agency is licensed first, then surveyed. Once you enroll your first client, you notify the HHSC regional office and request your initial health survey (Form 2020), which must happen within six months of your license date. So you can be licensed and serving private-pay clients before your first on-site survey — plan to be survey-ready from day one anyway.

4.3 Timeline

Budget a couple of months from a complete TULIP submission to license, plus the time to write your policies and finish the training. Then your initial survey follows your first client, within six months. Medicaid enrollment and MCO contracting (Part 8) run on their own, longer track — which is why private pay comes first.

PART 5

The Administrator, Supervisor & Attendants — by Position

In short: Texas is specific about your administrator's training and your background checks, and requires a service supervisor. Here it is, one role at a time — and the administrator training is the long pole.

5.1 Your administrator & alternate administrator (26 TAC §558.244, §558.259–260)

Every HCSSA must have an administrator, plus an alternate administrator to cover absences (who may be you and a partner). Here's exactly what it takes:

- 1. Meet the qualifications.** An administrator must have a high-school diploma/GED plus at least one year of experience or training in caring for people with functional disabilities; OR two years of full-time college in a health-related field; OR qualify as a licensed home health/hospice administrator.
- 2. Complete the 24-hour initial training.** A first-time administrator AND the alternate administrator must each complete 24 clock hours of initial management training — structured as 8 hours BEFORE designation and 16 hours within the first 12 months after. There's no exemption for nurses or prior experience.
- 3. Keep up continuing education.** 12 clock hours of continuing education each 12-month period from designation, covering the required topics.
- 4. Name your alternate.** The alternate administrator must meet the same qualifications and be able to act when the administrator is unavailable.

Start the administrator training early. The 8 hours must be done BEFORE you designate the administrator, and you can't be licensed without one. Line up an approved HCSSA administrator course before you apply.

5.2 Your service supervisor (26 TAC §558.404)

A PAS agency doesn't need a full-time supervising nurse the way a skilled home health agency does — but it does need a service supervisor to oversee care. A service supervisor must be a licensed nurse, OR have two years of full-time study at an accredited college/university (a person with a high-school diploma/GED may substitute one year of full-time supervisory employment in a health-care facility or community-based agency for one year of the college study). The supervisor makes the initial on-site visit to set up each client's care and enforces your written supervision policy.

5.3 Your attendants / caregivers

- **Competency, not a fixed hour count:** Texas requires attendants to have demonstrated competency for the tasks they're assigned; it does not mandate a specific number of pre-service training hours for general PAS attendants (a specialized task like G-tube feeding has its own strict training and testing rules). Build a solid orientation and skills check-off anyway — it protects clients and wins referrals.
- **Orientation (§558.245):** every employee is oriented to your policies, procedures, and objectives, and direct-care staff sign that they've read and will follow your policies; provide ongoing staff development.
- **Supervision:** follow your written supervision policy, developed with input from the client/family on how often a supervisor checks in, with the required initial on-site supervisory visit documented.

Confirm TB/health screening. The PAS rules center on competency and infection-control policies; a specific pre-employment TB test isn't clearly spelled out for PAS attendants in Chapter 558. Confirm current TB-screening expectations with HHSC and follow DSHS guidance — many agencies screen anyway as a best practice.

5.4 Background checks — the system you must run

- 1. Search the EMR and NAR before first client contact.** Before an unlicensed employee, volunteer, or contractor has first face-to-face contact with a client, search BOTH the Employee Misconduct Registry and the Nurse Aide Registry to confirm they're not listed as unemployable — the free HHSC online search.
- 2. Repeat the registry searches annually** for every employee — and never employ anyone listed as unemployable or found to have abused, neglected, or exploited a client.

3. Run a criminal-history check. Check Texas DPS criminal history on unlicensed applicants; Health & Safety Code Chapter 250 (§250.006) lists the convictions that bar employment. Use the DPS Crime Records Service or a background-check vendor for broader/out-of-state records.

4. Document everything. Keep the dated registry and criminal-history results in each employee file — a surveyor will ask to see them.

5.5 Recruiting and keeping good caregivers — the real make-or-break

Every experienced agency owner will tell you the same thing: your business rises or falls on caregivers, not clients. Referral sources will send you all the clients you can handle — but only if you can reliably staff them. So treat recruiting and retention as your core operation, not an afterthought.

- **Recruit constantly:** post continuously (Indeed, local Facebook groups, word of mouth, and a caregiver-referral bonus), and make applying fast — the best caregivers take the first agency that calls them back and schedules an interview.
- **Hire for reliability and warmth:** specific skills can be trained; showing up on time and being kind cannot. Check references for dependability, not just experience.
- **Onboard properly:** a real orientation, a skills check-off before a caregiver works alone, and a written scope of what they may and may not do — this protects clients and builds the caregiver's confidence.
- **Retain deliberately:** consistent hours, quick and correct pay, being reachable when a caregiver has a problem, genuine recognition, and a small raise ladder. Turnover is the biggest hidden cost in home care — every caregiver who quits costs you recruiting, training, and often the client they served.

PART 6

EVV, Policies & Getting Operational

In short: Write your policies, set up Electronic Visit Verification for Medicaid visits, and get ready for your first-client survey. EVV is a hard gate to Medicaid payment, so set it up before you take a Medicaid client.

6.1 Your policies & procedures (the P&P Manual)

HHSC expects a governing body and written policies covering client rights, admission/eligibility, the service plan and supervision, complaint handling, emergency preparedness, infection control, personnel and background checks, incident reporting, and abuse/neglect reporting. This is your operating backbone and what the surveyor reads first — a complete P&P Manual (and matching forms) is the single biggest time-saver at survey.

6.2 Set up EVV (before your first Medicaid visit)

Electronic Visit Verification is mandatory for Medicaid personal-care visits and is the gate to getting paid — a claim that doesn't match a verified EVV visit won't be paid.

1. **Pick your EVV system.** Use the free statewide HHAeXchange portal, or get HHSC approval to use an alternative (proprietary) EVV system that meets state requirements.
2. **Onboard and train.** Complete the required EVV onboarding and training, and set up your caregivers, clients, and authorizations in the system.
3. **Capture every visit.** Each visit records who/what/where/when; use the correct reason codes and complete "visit maintenance" to fix exceptions.
4. **Bill against verified visits.** Claims flow through the TMHP EVV Aggregator — the visit must be verified before the claim pays. Expect EVV compliance reviews from HHSC and the MCOs.

Private pay is EVV-free. EVV applies to Medicaid personal-care services. If you launch on private pay and long-term-care insurance, you don't need EVV until you take your first Medicaid client — one more reason private pay is the easier on-ramp.

6.3 Be ready for your first-client survey

Once you enroll your first client, notify the HHSC regional office and request your initial health survey (Form 2020) — it must occur within six months of licensure. The surveyor checks your policies, personnel files (training and background checks), client records and service plans, and supervision documentation. Keep every file complete and current from day one and the survey is straightforward.

6.4 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 7

The Client's Journey — Intake to Discharge

In short: A client is referred (privately or by an MCO), you assess and set up the service plan, an attendant delivers the hours (with EVV for Medicaid), a supervisor oversees, and you review and adjust. Here is the whole journey, stage by stage.

Stage 1 — Referral & inquiry

A referral comes from a family, a hospital discharge planner, a home-health/hospice partner, or a STAR+PLUS MCO service coordinator (Part 10). Someone asks whether you can staff care for a specific person.

What you do:

- Take the basics (who needs care, what tasks, how many hours, where, when to start, and who pays — private, Medicaid MCO, VA, or long-term-care insurance). Confirm the need is non-medical and within your PAS scope.

Stage 2 — Assessment & the service plan

For every client, do an in-home assessment and build a service plan: the specific tasks (bathing, dressing, meals, housekeeping, transport, companionship), the schedule and hours, and any safety concerns. For Medicaid clients, the MCO's assessment and authorization set the approved hours and services; your plan aligns to it.

Stage 3 — Agreement & start of care

The client (or representative) signs a service agreement covering the services, schedule, rate/payer, client rights, and how service can end. The service supervisor makes the required initial on-site visit to set up care, and you match a screened, competent attendant to the client.

Stage 4 — Delivering the hours (with EVV for Medicaid)

The attendant delivers the scheduled care; for Medicaid visits, each one is captured in EVV (clock-in/out, tasks, location). Keep visit notes, and make sure caregiver skills match the client's needs — consistency of caregiver is the number-one driver of client satisfaction and retention.

Stage 5 — Supervision & quality

Follow your written supervision policy: the supervisor checks in on the schedule you set with the family, monitors the attendant's performance and the client's changing needs, and documents it. Run your quality program — client satisfaction, complaint handling, incident reports — and report any suspected abuse, neglect, or exploitation.

Stage 6 — Changes, reassessment & authorizations

As the client's needs change, update the service plan; for Medicaid clients, work with the MCO service coordinator on reassessments and new authorizations (hours can go up or down). Keep authorizations current — delivering unauthorized hours doesn't get paid.

Stage 7 — Discharge & transition

When care ends — recovery, a move to a higher level of care, or a hospice transition — follow your discharge policy, give proper notice, coordinate with the family and any MCO, and document the transition. A good discharge (and a follow-up call) turns former clients and their families into referral sources.

PART 8

How You Get Paid — Private Pay, Medicaid, VA & LTC Insurance

In short: Start on private pay and long-term-care insurance the day you're licensed; add Medicaid (STAR+PLUS and the attendant programs) once you enroll and contract. Non-medical home care is paid by the hour.

Your four revenue streams:

Source	What it is	Key fact
Private pay	Client/family pays out of pocket, hourly	Start immediately on your license; highest rate; no EVV/Medicaid enrollment
Medicaid (STAR+PLUS etc.)	Managed long-term care + attendant programs	The biggest, steadiest market; requires enrollment, MCO contracts & EVV
Long-term-care insurance	The client's LTC policy reimburses in-home care	Bill the family or the insurer once benefit triggers are met
Veterans (VA)	VA Homemaker/HHA, Veteran-Directed Care, Aid & Attendance	Serve veterans via a VA contract / Community Care Network, or A&A-funded private pay

8.1 Private pay — your launch market

The day your license issues, you can serve private-pay clients — families paying out of pocket, typically billed by the hour with a visit minimum (often 3–4 hours). In Texas, non-medical homemaker care runs roughly \$28–\$30/hour to the client (2024–2025 cost-of-care surveys); you set your own rate above your fully-loaded caregiver cost. No EVV, no Medicaid enrollment, no MCO contract — just a clear service agreement and good caregivers. Long-term-care insurance is part of this stream: many clients have an LTC policy that reimburses in-home personal care once ADL/cognitive benefit triggers are met — you bill the family or the insurer.

8.2 Medicaid — the bigger market (STAR+PLUS & the attendant programs)

Most Texas Medicaid personal care runs through STAR+PLUS, the managed long-term-care program for adults 65+ and adults with disabilities; there's also Community First Choice (a state-plan personal-care benefit) and the legacy attendant programs — Primary Home Care (PHC), Community Attendant Services (CAS), the state-funded Family Care program, and the CLASS waiver. Across all of them, clients can choose the Consumer Directed Services (CDS) option (the client is the employer) — and your agency can serve CDS clients by also enrolling as a Financial Management Services Agency (FMSA), a second revenue line.

How to get set up to bill Medicaid

- 1. Hold your HCSSA-PAS license first (Part 4).**
- 2. Enroll with Texas Medicaid via TMHP/PEMS.** Use the Provider Enrollment and Management System; enroll your NPI with the personal-care taxonomy (3747P1801X) under Acute Care fee-for-service — that record is what lets you contract with MCOs. TMHP: 800-925-9126.
- 3. Contract with the STAR+PLUS MCOs in your area.** Credential and contract with each plan separately: Wellpoint (formerly Amerigroup), Molina, Superior HealthPlan, and UnitedHealthcare span many areas; Community First Health Plans (Bexar/San Antonio), Community Health Choice (Harris/Houston), and El Paso Health serve their regions.
- 4. Set up EVV (Part 6)** before you deliver a Medicaid visit.

8.3 The rate & billing

Personal attendant services are paid per hour (per unit). HHSC sets the base methodology and MCOs pay at least the HHSC rate; effective September 1, 2025 the rates were built on an attendant base wage of \$13.00/hour, plus payroll taxes & benefits (14% non-residential) and a small per-hour administrative add-on. The exact billable per-unit rate lives in HHSC Provider Finance's schedules — pull the current figure there rather than relying on a quoted number. For timely filing, fee-for-service claims go to TMHP within 95 days of the date of service; for managed care,

you bill the MCO on its (typically 95-day) window. Remember: a Medicaid claim must match a verified EVV visit to be paid.

Ask each MCO:

- Your hourly rate for personal attendant services and how authorizations work; the service-coordination and claims portals; the EVV expectations; the timely-filing window; and how a reassessment changes authorized hours.

8.4 Veterans

- **VA Homemaker / Home Health Aide (H/HHA):** the VA's explicitly non-medical in-home program — aides work for an agency that holds a VA contract or is in the VA Community Care Network (administered regionally by TriWest). This is your entry point to serve veterans.
- **Veteran-Directed Care (VDC):** the veteran gets a flexible budget to arrange their own services and can hire caregivers — you can be their agency.
- **Aid & Attendance:** an increased VA pension for eligible wartime veterans/survivors that can be applied toward private in-home care — effectively making your private-pay service affordable to the veteran. Verify current amounts on VA.gov.

PART 9

What It Costs & How Long It Takes

In short: Budget the license, the administrator training, insurance & bonding, EVV, payroll float, and (optionally) accreditation. Plan a couple of months to license and start private pay, longer to add Medicaid.

9.1 Startup line items (typical — verify)

Item	What's in it	Ballpark
HCSSA-PAS license	3-year initial license (parent agency)	\$2,625
Administrator training	24-hr initial course for administrator + alternate	Several hundred \$ each
Business setup	SOS Form 205 \$300, EIN (free), NPI (free), DBA, legal	~\$300 + legal
Insurance & bond	GL + professional + abuse + non-owned auto + dishonesty bond (+ WC)	Get quotes — annual
EVV	HHAExchange (free) or an approved alternative system	\$0–modest
Software	Scheduling / EVV / billing / payroll platform	Monthly SaaS
Payroll float	You pay caregivers weekly but collect from payers later	Working capital — plan for it
Policies	P&P Manual + Forms Packet	One-time
Accreditation (optional)	ACHC / CHAP / Joint Commission (see 9.2)	~\$7,000–\$14,500 / 3-yr cycle

The cash-flow trap to plan for. Home care is payroll-heavy: you pay caregivers weekly, but Medicaid and insurers pay you weeks later. Private-pay clients (billed in advance or on short terms) smooth this out — another reason to launch on private pay. Line up a business line of credit before you scale Medicaid hours.

9.2 If you pursue accreditation

- **Why:** some STAR+PLUS MCOs, VA contracts, long-term-care insurers, and franchise brands prefer or require accreditation, and it's the on-ramp if you later add Medicare-certified skilled home health.
- **Who:** ACHC, CHAP, and The Joint Commission all accredit home care; expect roughly \$7,000–\$14,500 over a 3-year cycle plus staff time, on a survey-then-3-year-term model.
- **When:** most non-medical agencies launch on the state license and add accreditation in year two once volume justifies it.

9.3 Timeline

Early	Entity, training, policies	Form the LLC, start the 24-hr admin training, write your P&P
Weeks	Insurance & pre-survey CBT	Bind coverage & bond; finish the free HHSC pre-survey training
~1–2 mo	TULIP application → license	Apply with documents + \$2,625; HHSC processes (up to 45 days)
Right away	Start private pay	Serve private-pay & LTC-insurance clients on your license
Months	Enroll & contract Medicaid	TMHP/PEMS + STAR+PLUS MCO contracts + EVV setup
~6 mo	Initial survey	Request the Form 2020 survey after your first client

9.4 The money math — a simple break-even

Home-care economics are simple once you see them laid out. On each private-pay hour, you bill the client a rate; out of that you pay the caregiver's wage plus payroll taxes, workers' comp, and your overhead. The gap is your gross margin — often roughly 30–40% of the bill rate for a well-run private-pay agency. A worked example, using round numbers you'll replace with your own:

- **Bill rate to the client:** say \$29/hour (set yours from your local market and what referral sources see).
- **Fully-loaded caregiver cost:** the wage (say \$15) plus about 15–20% for payroll taxes, workers' comp, and paid time — roughly \$18/hour all-in.
- **Gross margin:** about \$12/hour, before overhead — from which you cover insurance, software, office, marketing, and your own pay.
- **What this means for you:** you need enough billed hours each week to cover your fixed overhead before you earn a profit — which is why census (total authorized client hours) and caregiver utilization are the two numbers to watch, and why a few reliable, long-hours clients are worth more than many tiny visits.

PART 10

Getting Your First Clients

In short: Clients come from discharge planners, home-health/hospice partners, care managers, senior communities, the Area Agencies on Aging and ADRCs, and — once contracted — the MCO service coordinators. Build these relationships before you open.

10.1 The referral sources

- **Hospital discharge planners & case managers:** the single highest-volume source for post-hospital home support — introduce yourself, leave a one-pager, and make it easy to refer to you same-day.
- **Skilled home-health & hospice agencies:** they refer out the non-medical/companion hours they don't provide; build reciprocal partnerships (you refer skilled needs to them).
- **Geriatric care managers & elder-law attorneys:** private Aging Life Care professionals place clients with vetted agencies; elder-law and estate attorneys advise families arranging care.
- **Senior living communities:** assisted-living, independent-living, and memory-care communities refer supplemental one-on-one care for their residents.
- **Area Agencies on Aging & ADRCs:** Texas has 28 AAAs (1-800-252-9240) and the Aging & Disability Resource Centers / “No Wrong Door” (855-937-2372) that field families looking for in-home help — get listed and known.
- **2-1-1 Texas & MCO service coordinators:** list your services with 2-1-1 Texas (dial 211), and once contracted, STAR+PLUS MCO service coordinators route member hours to in-network agencies.

10.2 What to ask each referral source

“How do you decide which home-care agency to refer a family to?”

“What do you need from me to be on your referral or preferred-provider list — license, insurance, bonding, Medicaid enrollment, availability?”

“Who exactly is the discharge planner, social worker, or case manager I should build a relationship with?”

“How fast do you need us to be able to start care after a referral — same day, 24 hours, 48 hours?”

“What makes an agency easy to work with, and what gets one dropped from your list?”

Speed and reliability win home-care referrals more than anything else: be the agency that answers the phone on the first call, can start care within 24–48 hours, communicates back to the referrer, and never no-shows a shift. One reliable start earns you the next ten referrals.

10.3 Your first-90-days referral plan

Referrals don't happen by accident — they come from a handful of relationships you build on purpose. Here's a simple plan for your first 90 days:

1. **Make a target list.** Write down the 10–15 hospital discharge planners, skilled home-health and hospice liaisons, senior-community directors, and case managers in your service area — these are the people who send families to agencies.
2. **Show up in person with a one-pager.** A clean one-page sheet: who you are, what you do, your service area, how fast you can start, your insurance/credentials, and your phone number. Leave it; then follow up a week later.
3. **Be absurdly responsive.** Answer every call, say yes to same-day starts whenever you can, and report back to the referrer after the first visit — that single habit earns the next referral.
4. **Ask for feedback and the next referral.** After a good start, ask the referrer what would make you easier to work with, and whether they have another family who needs help. Then do it again next week.

PART 11

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your master contact + verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

Licensing, the administrator & staff

Who	Contact	What to verify / ask
HHSC — HCSSA Licensing	hhs.texas.gov (HCSSA) · TULIP portal	The pre-survey training; the application (Form 2021); the current fee; the initial survey (Form 2020)
HHSC — background-check registries (EMR/NAR)	hhs.texas.gov (EMR/NAR search)	How to run the free EMR & NAR searches before hire and annually
Texas DPS — Crime Records Service	dps.texas.gov (Crime Records)	Criminal-history checks; the HSC Ch. 250 disqualifying convictions
HCSSA administrator training provider	HHSC-recognized course vendors	The 24-hour initial training + the 12-hour annual CE
Texas Board of Nursing	bon.texas.gov	RN delegation rules if attendants do any health-related tasks

Business, insurance & billing IDs

Who	Contact	What to verify / ask
Texas Secretary of State	(512) 463-5555 · sos.state.tx.us	Forming your LLC (Form 205, \$300); assumed name (DBA)
Texas Comptroller	comptroller.texas.gov	Franchise-tax good standing & the Public Information Report
IRS	irs.gov (EIN Assistant)	Get your EIN (free)
NPPES / CMS	nppes.cms.hhs.gov	Your Type 2 NPI and the personal-care taxonomy (3747P1801X)
Insurance agent (home-care program)	trustedchoice.com ; carriers in Part 3.4	GL + professional + abuse + non-owned auto + dishonesty bond

Funding, EVV & referrals

Who	Contact	What to verify / ask
TMHP — provider enrollment (PEMS)	800-925-9126 · tmhp.com	Enrolling your NPI/taxonomy; the Acute Care FFS record for MCO contracting
STAR+PLUS health plans (MCOs)	Wellpoint, Molina, Superior, UnitedHealthcare, Community First, Community Health Choice, El Paso Health	Which serve my area; contracting; the hourly rate; authorizations; timely filing
HHSC Provider Finance (rates)	pfd.hhs.texas.gov · PFD-LTSS@ hhs.texas.gov	The current personal-attendant per-hour rate
HHSC EVV / HHAeXchange	hhs.texas.gov (EVV) · hhaexchange.com/texas	EVV onboarding, training, and compliance
VA Community Care / TriWest	va.gov/communitycare · tricare-west (TriWest)	The VA Homemaker/HHA program; joining the Community Care Network

Referral & senior-services partners

Who	Contact	Role
Area Agencies on Aging (AAA)	1-800-252-9240 · hhs.texas.gov (AAA)	Benefits counseling & in-home-care referrals for adults 60+
Aging & Disability Resource Centers	855-937-2372 (855-YES-ADRC)	The “No Wrong Door” access point for long-term services & supports
2-1-1 Texas	dial 211 · 211texas.org	Free statewide referral line — list your services
Accreditor (optional) — ACHC / CHAP / TJC	achc.org · chapinc.org · jointcommission.org	Whether/when to accredit; cost quotes; payer requirements

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

You can do this

Opening a Texas non-medical home care agency is one of the more accessible care businesses to start — no building, no nurse-owner, and you can earn on private pay the day you're licensed. Take it one rung at a time: form the business, get the HCSSA-PAS license, train your administrator, recruit and screen great attendants, launch on private pay and long-term-care insurance, add Medicaid through TMHP and the STAR+PLUS plans, and build referral relationships with discharge planners and the aging network. Verify each fee and contact as you go, and lean on the people in Part 11.

Reminder. Prepared for PolicyReady.org and grounded in Texas Health & Safety Code Ch. 142, 26 TAC Chapter 558, and current HHSC/Texas Medicaid information as of 2026. This is general information, not legal, tax, or billing advice. Requirements, fees, rates, and contacts change — verify each with HHSC, TMHP, the STAR+PLUS health plans, and qualified counsel before you rely on it. Confirm the current license fee, the personal-attendant rate, and your EVV setup before you rely on them.